APIXABAN

Twice daily

Not for mechanical valves

CHEMICAL NAME 1-(4-methoxyphenyl)-7-oxo-6-[4-(2-oxopiperidin-1-yl)phenyl]-4,5,6,7-tetrahydro-1H-pyrazolo[3,4-c]pyridine-3-carboxamide

CLASS Direct oral anticoagulant (DOAC) — direct factor Xa inhibitor

TRADE NAMES Eliquis · Apixa

THERAPEUTIC USES / INDICATIONS

- Stroke prevention in non-valvular atrial fibrillation (lowest bleeding among DOACs in trials).
- Treatment of DVT/PE and prevention of recurrence.
- VTE prophylaxis after hip/knee replacement.

ADMINISTRATION

- **PO** twice daily with or without food.
- Can be crushed and given in water, apple juice or via NG tube.

DOSAGES

- **AF:** 5 mg twice daily; 2.5 mg twice daily if ≥2 of: age ≥80 yr, weight ≤60 kg, creatinine ≥133 µmol/L (1.5 mg/dL) (or CrCl 15–29).
- **DVT/PE:** 10 mg twice daily for 7 days, then 5 mg twice daily; extended prevention 2.5 mg twice daily after 6 months.
- **Hip/knee replacement:** 2.5 mg twice daily (start 12–24 h post-op).
- Avoid if CrCl <15.

PHARMACOKINETICS

- **Onset:** 3–4 h (peak). **Duration:** ~12 h.
- Bioavailability ~50%; metabolised by CYP3A4; P-gp substrate.
- Excreted ~27% renal, rest faecal. **t½:** 12 h.

PHARMACODYNAMICS

- Directly and reversibly inhibits **free and clot-bound factor Xa** → ↓ thrombin generation.
- Indirectly inhibits platelet aggregation induced by thrombin.

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Bleeding (nose, gums, bruising).
- Nausea.
- Anaemia.

ADVERSE EFFECTS (SERIOUS)

- Major bleeding (GI, intracranial).
- Spinal/epidural haematoma.
- Hypersensitivity.
- Reversal: andexanet alfa or PCC.

CONTRAINDICATIONS & CAUTIONS

- Active significant bleeding.
- Mechanical heart valves; significant mitral stenosis.
- Hepatic disease with coagulopathy.
- Pregnancy, breastfeeding.

KEY INTERACTIONS

- **Strong CYP3A4 + P-gp inhibitors:** avoid (or reduce dose).
- **Rifampicin, carbamazepine, phenytoin, St John's wort:** avoid.
- **Antiplatelets, NSAIDs, SSRIs:** ↑ bleeding.

NURSING MANAGEMENT

- Calculate correct dose using age, weight and creatinine criteria.
- Baseline and periodic renal function, LFTs, CBC.
- Monitor for bleeding.
- Peri-procedure stop/restart plan.
- Reinforce twice-daily adherence.

PATIENT EDUCATION

- Take morning and evening, 12 h apart.
- Missed dose: take as soon as remembered on the same day, then continue twice daily.
- Report bleeding; carry alert card.
- Do not stop without advice (stroke risk).

KEY POINTS

- Direct Xa inhibitor, twice daily.
- VTE: 10 mg BD × 7 days.
- AF 5 mg BD (2.5 mg if 2 of: ≥80 y, ≤60 kg, Cr ≥133).
- Not for mechanical valves; reverse with andexanet.